Shoulder Arthroscopy *Prodigy Anesthesia*

Arthroscopic shoulder surgery is routinely performed for procedures such as debridement of labral tears, distal clavicle resection, subacromial decompression, Bankart repairs for recurrent shoulder dislocation, capsular release, rotator cuff repair, and repair of superior labral anterior-posterior (SLAP) tears. There is less pain and a faster recovery from arthroscopic shoulder surgery than open surgery due to less trauma to the tissues.

The procedure is usually performed in the lateral or semi-sitting (beach chair) position. If placed in the lateral position, the patient will have an axillary roll underneath the dependent portion of the thorax and traction weights will be applied to the surgical arm. The 'down' arm is placed in forward flexion. There is a risk of venous air embolism in the beach chair position, but the most common risk due to this type of positioning is postural hypotension. The patient should gradually be eased into position to prevent sudden drops in blood pressure. Antiembolism stockings and adequate hydration prior to positioning is also recommended.

Because even slight bleeding can obscure the arthroscopic view of the shoulder, the mean arterial pressure is maintained less than 80 mmHg to minimize bleeding. Epinephrine is also typically instilled into the irrigation fluid (1 mg/3L) to decrease bleeding.

Many practitioners utilize a rigid eye patch to protect the ipsilateral eye during the surgery. Occasionally, procedures are performed under regional anesthesia alone, but general endotracheal anesthesia with a brachial plexus block for postoperative pain is the most common anesthetic method.

Jaffe RA, Samuels SI. Anesthesiologist's Manual of Surgical Procedures. 4th ed. Philadelphia, PA: Lippincott Williams and Wilkins, 2009: 936-947.

Copyright 2014 Prodigy Anesthesia, Inc. It is unlawful to copy, distribute, and/or sell this material.